

[Indexed as: **Delange v. Parkinson Estate**]

**Ray Delange, Yvonne Delange, Plaintiffs and The Estate of
Marilyn Parkinson, Deceased and Derek Parkinson, Defendants**

Ontario Court of Justice, General Division

Brockenshire J.

Judgment: August 13, 1997

Docket: Windsor 92-GD-21976

Ron Shulgan and James Cooke, for the Plaintiffs.

David G. Greenaway, for the Defendants.

ACTION by plaintiffs for damages arising from motor vehicle accident.

Brockenshire J.:

- 1 This is a claim for damages arising from a motor vehicle accident on July 7, 1990. The plaintiff, Ray Delange, was injured in the accident. His claims relate to his personal injuries, out-of-pockets, and past and future income losses. His wife, Yvonne, and his children, Andrew and George, claim for damages under the *Family Law Act*.
- 2 The issues before the court were liability, the “threshold” and quantum of various heads of damages. The quantum of benefits to be deducted from a recovery, pre-judgment interest, and costs, are to be addressed later.

Liability

- 3 This issue can be dealt with very briefly. The uncontradicted evidence of Ray Delange is that at 7:10 a.m. on Saturday, July 7, 1990, he was driving westerly on Kent County Road 8, the old Highway 98, towards Tilbury, at 80 kms. per hour, the speed limit, wearing his seat belt, when his unremarkable journey was tragically interrupted by a blue car suddenly appearing before him in his lane. He tried to brake and turn but did not have sufficient time or room. He could not avoid the collision, which occurred in his lane. The blue vehicle had been travelling south on County Road 5, which was clearly marked as a stop street for County Road 8. There was no evidence as to whether the blue car stopped at all for the stop sign but even if it had, it clearly entered County Road 8 in the face of the immediate hazard of the plaintiff's approaching vehicle. It is admitted that Marilyn Parkinson, who was killed in the accident, was the driver, and Derek Parkinson was the owner of the blue vehicle. I find on the evidence that the entire responsibility for the accident and any resulting damages, lies upon the defendants.

Personal Background of the Plaintiff

- 4 Ray Delange is now 48 years of age. He immigrated from Holland many years ago. For the past 27 years, he has been a heavy construction contractor, starting on his own with one machine and two employees, and growing until, in 1990, he had 10 pieces of heavy equipment, all paid for, 25 employees, no debt, and money in the bank. By 1990, he was the biggest heavy construction contractor in Kent County, doing mainly sewer and water line work. He has been married to Yvonne for 26 years. Their two sons are now in the 20's. One has an engineering degree, the other a certificate in engineering technology. The wife and sons work in the family business. Before the accident Ray Delange was a very energetic, hard-working man, who typically, in good weather, would leave home for work at 6:00 a.m., coming home at 9:00 p.m., and then working on bids and estimates.

His original trade was carpentry, and he maintained, and added 4 bedrooms to, his own home. On the job, he would do everything from supervising, to operating the machines, to working as a labourer beside his men. He was described as a sociable, jolly fellow, who could make people laugh. His business was doing well, and he had vague plans of retiring early, when the money was right, and when his sons could take over the business.

His Injuries

- 5 Ray Delange suffered three fractures of the sternum, probably from hitting the steering wheel. He had severe bruising to the left thigh, left shoulder and left knee. He was admitted to hospital, and held for several days because of the possibility of cardiac contusion. He developed, in addition to his severe chest pain, and pain to the other bruised areas, pain in his neck and headaches. He was extremely fatigued. In the next few months, the bruised areas cleared up, the fractures healed, but the chest pain persisted, and seem to radiate to the back of his chest, the neck pain and stiffness continued, with the pain spreading to the left scapular area, the headaches persisted, and he could only sleep about 4 hours per night. He was trying to work, but could only last a few hours.

Present Complaints

- 6 He no longer complains of headaches, saying they let up after two years. His sleep problem continues, and he has to take medication to get to sleep, sleeps little and is fatigued. He gets pain in the front of his chest and breathing problems if he tries to lift anything heavy. Otherwise, the front of his chest is pain free. His continuing problem is pain in his neck and back which he describes as a pulling sensation, and as continuous discomfort. In cross-examination, he described the neck-back pain as extending down to a point roughly 4 inches above his belt and out into his shoulders, particularly his left shoulder. Additionally, he complained of a painful or pulling sensation in his left side.
- 7 In cross-examination it was put to him, that he had told some doctors, that additionally, he had low back pain, with the first such complaint being in May of 1993. He felt the low back pain related to the accident, and explained the delay in his reporting it as masking of the pain by medication the first few years after the accident, and by that pain being much less than that in his thoracic back and his neck.

Effects on Work and Leisure

- 8 Mr. Delange testified that he is now constantly in pain and fatigued. He can no longer open or climb into manholes, crawl in ditches, walk over rough ground, bend and twist, lift, operate the machines, do heavy labour, or even drive for a long distance. He still goes out to the job sites but only for a few hours a day, and when he is there he can only stand or sit and watch. He still does the estimating work in the office, but cannot do the field inspections he used to do in preparing bids. He is no longer able to take care of his lawn, garden and house repairs, let alone undertake renovations at home. He can no longer drive or sit in a car for hours at a time, so no longer feels like going on vacations. He has flown south a couple of times, to avoid the drive. He appreciates that he is irritable, and that this affects his family life. His social life is drastically reduced. All of this was supported by the evidence of his wife, and several workers, and not contradicted by any defence eye witnesses or videos.

Medical Opinions

- 9 Dr. Tomen, the family doctor, has treated Mr. Delange continuously since the accident, seeing him about 45 times. He testified that, in his view Mr. Delange has a chronic pain condition and fatigue, with associated mood changes, all of which relate directly to the accident, and which plateaued 4 or 5 years ago. He has tried every treatment available, without success, and in Dr. Tomen's opinion the condition is permanent and seriously interferes with his work and leisure. He does not see him as magnifying his symptoms.
- 10 Dr. Patterson, a surgeon, saw Mr. Delange on referral from Dr. Tomen on September 18, 1990, when he was having chest pain from the fracture site radiating into his back, between his shoulder blades. Dr. Patterson thought this was appropriate for the injury and expected an eventual full recovery. He did not see Mr. Delange again.
- 11 Dr. Varkey, of the London University Hospital Pain Clinic, saw Mr. Delange 6 times and gave local anaesthetic and laser treatments for what he described as chronic myofascial pain. He did not give an opinion on prognosis. Mr. Delange had said that the treatments provided temporary help, but each time the pain returned.
- 12 Dr. Stuart Young, a surgeon, did a medical legal examination on October 7, 1996 and a subsequent report on behalf of the plaintiff's counsel, and testified at the trial. The complaints made to him were of first, left shoulder pain, and below the shoulder blade, spreading up to the left shoulder and

cervical area, which is constant and aggravated by lifting and bending; second, occasional pressure sensations (2 or 3 times a week) at the site of the old sternum fracture; third, neck stiffness, daily, and fourth, low grade low lumbar pain. Dr. Young found on a physical examination, pressure and percussion on cervical joints produced local pain, pressure on the 11th rib produced pain just as Mr. Delange had described it, and percussion at L5 and S1 produced local pain. He described the serratus posterior inferior muscle, connected to the lower ribs, and he opined that spasm in that muscle produced the peculiar band-like pain in the low chest area described by Mr. Delange. His conclusion was that Mr. Delange had permanent ligaments or soft tissue injuries arising from a moderately severe whiplash-type injury, plus residual cartilage and muscle injuries in the chest, relating back to the sternum fracture. In his view the resulting symptoms are chronic and permanent.

13 Dr. Finestone, a specialist in physical medicine and rehabilitation and an associate professor at Western did a medical legal examination October 22nd 1996 and a subsequent report at the request of Plaintiff's counsel. His testimony at trial, amplified by his report, was of receiving complaints as follows:

- 1) left paraspinal area pain radiating from the left mid-paracervical region down to the lower back with a constant pulling type sensation from a specific area on the left side of his spine which would worsen with activity. Headaches would frequently be associated with that pain and so was a lack of stamina.
- 2) Low back pain described as a strained pulling sensation in the right and left low back region, referred to as aggravating discomfort but nowhere near the paraspinal pain and;
- 3) Chest pain in the inferior sternal area brought on by activity. Palpation did not produce tenderness in the sternum area but did produce significant discreet tenderness in the upper left trapezius, over the left posterior inferior aspect of the rib cage and lateral to C7, bilaterally. There was tenderness noted along the inferior left parathoracic musculature. A response was elicited at only three of the eighteen fibromyalgic tender points.

14 His diagnosis was that he does not suffer from fibromyalgia but does have what could be called a chronic pain syndrome, which could also be referred to as cervical thoracic and lumbar sprain or strain or myofascial pain. His opinion was that this was caused by a biomechanical force applied to his spinal region leading to tissue disruption, that the same would apply

to the low back, perhaps with some contributing degenerative changes, and that the chest pain arose from the impact sustained on his chest and secondary soft-tissue injury. He attributes all of this to his motor vehicle accident of July 7, 1990.

- 15 He considered the opinion of Dr. Young relating to the serratus muscle and felt that he could not say that the pain in that area could be referred solely to that muscle.
- 16 He felt there could be further improvement re. the chest pain but the chances of further improvement in the other pains were unlikely. He would continue to be restricted from prolonged physical activity and certainly could not work as hard and as long as he used to.
- 17 In cross-examination it was put to him that Dr. Clifford had suggested it might be beneficial to "work through the pain". Dr. Finestone disagreed, opining that that philosophy fitted some injuries very well, such as sports injuries but should not be universally applied.
- 18 Dr. Clifford, a specialist in physical medicine and rehabilitation saw Mr. Delange on referrals from Dr. Tomen in 1991 and in 1992 and then reassessed him October 18, 1996. His reports were made exhibits, along with the reports of all the other doctors, and he testified, being called as a defence witness. The symptoms described to him in 1991 and 1992 were of chronic neck pain and chronic anterior chest pain, with the neck pain extending into the right trapezius muscle. In 1991, he recommended further physiotherapy. That took place but the 1992 follow-up showed no improvement. The 1996 reassessment revealed diffuse discomfort along the left paracervical, parathoracic and paralumbar musculature.
- 19 His conclusions in evidence were that the pain did not arise from some undiagnosed pathology and he did not see any indication of dissimulation. His impression was that he had bona fide chronic pain or that he had a "valid belief system" meaning as I understand it that the patient may validly believe they are suffering from an organic problem which causes a reduction in pain threshold.
- 20 The theory of Dr. Clifford was that one could assume the underlying physical injury had healed but due to scar tissue etc. it may well not be exactly as before. He felt that a return to full physical activity would not do physical harm to Mr. Delange and that although it would be painful, eventually the perception of pain would reduce. He expected Mr. Delange would continue to report the pain into the foreseeable future but the quality and quantity of it might alter over time.

- 21 In his 1992 report to Dr. Tomen he had summed up the situation by saying that the ongoing pain perception was real and there was unlikely to be a quick cure, there was undoubtedly no acute ongoing tissue pathology, increasing physical activity would be safe but may well cause exacerbations of pain but the long-term benefits of physical activity are substantial both physically and psychologically.
- 22 Dr. Yovanovich is an orthopaedic surgeon who examined Mr. Delange and reported thereon on February 7, 1995 for the defence. He testified that the complaints reported to him, in order of severity were a left-sided thoracolumbar pain, which was described as a pulling sensation and like a toothache, which continues daily but tends to be intermittent lasting a few hours to the entire day, a lumbar low back pain which was described as constant and worse in damp cold weather, cervical or neck pain described as intermittent occurring on a daily basis and occasionally radiating centrally down to his mid-back or interscapular areas, which tend to be aggravated but sudden movements or straining, and sleep difficulties and fatigue. Mr. Delange described all of these as continuing since the accident. As Dr. Yovanovich notes the various reports do not show reference to low-back symptoms until 1993.
- 23 Dr. Yovanovich noted Mr. Delange to be cooperative and forthright and he sensed no evidence of magnification of symptoms.
- 24 On physical examination he found a mild and diffused tenderness over most of the left trapezius muscle but none in the cervical area. He found no tenderness nor percussion pain over the left flank nor over the entire left side of his back and no lumbar tenderness. X-rays showed degenerative changes in the L5-S1 disc space. A CT scan showed some abnormality in the left kidney region.
- 25 The conclusions of Dr. Yovanovich were that this was a rather complex problem. He felt there had been a complete recovery from the sternum fractures. He felt it was clear that there had been symptoms of neck pain going back to the accident time and concluded Mr. Delange suffered soft tissue injuries to his neck and upper back and continues to experience mild and aggravating cervical pain. He did not feel there was any clear evidence that the flank pain or the lumbar pain related to the accident. He felt the left thoracolumbar pain could result from injury to the muscles between his ribs but might also indicate some kidney problem. His opinion was that the low back pain symptoms are the result of degenerative disease at L5-S1.
- 26 He felt that the lumbar pain would likely be chronic with exacerbations but that he could learn to live with them, that the cervical pain was mild and

unlikely to cause functional problems and concluded it would be perfectly safe for Mr. Delange to resume all of his activities. In his opinion Mr. Delange did not suffer any serious injury which would result in permanent impairment of a bodily function. Further he saw no indication that Mr. Delange suffered any physical impairment which would substantially interfere with his ability to carry out his work or his activities of daily living. In cross-examination he admitted that an injury relating to the serratus muscle could explain the thoracolumbar symptoms.

The Threshold

- 27 The defence had pleaded, and at trial relied upon s. 266 of the *Insurance Act*, which had come into effect a few days before this accident, which denies claims for loss or damage for bodily injury arising from the use or operation of an automobile unless the injured person has sustained “permanent serious impairment of an important bodily function caused by continuing injury which is physical in nature.”
- 28 The leading case on the interpretation of that section is of course *Lento v. Castaldo* (1993), 15 O.R. (3d) 129 (Ont. C.A.). As the headnote summarizes, the court has to ask itself a series of three questions:
- 1) Has the injured person sustained permanent impairment of a bodily function caused by a continuing injury which is physical in nature? And if yes then...
 - 2) Is the bodily function, which is permanently impaired, an important one? And if yes then...
 - 3) Is the impairment of the important bodily function serious?
- 29 The position of the defence was that it recognizes there is an impairment. The questions raised are whether the impairment is physical in nature and whether it is serious.
- 30 The defence looks to the evidence of Dr. Clifford, who said there was no ongoing acute pathology, so that from his system of definition, medically there would be no continuing injury. The view of Dr. Clifford had been that it could be assumed that the underlying physical injury had healed but due to scar tissue etc. it may well not be exactly as before.
- 31 In my view this is simply playing with words. To reduce this approach to absurdity, if a person’s legs were amputated, once the stumps had healed, one would have to say the person had no continuing physical injury.
- 32 Even Dr. Clifford did not see any indication of “dissimulation” and his impression was that Mr. Delange had bona fide chronic pain. His opinion

was that Mr. Delange would continue to report the pain into the foreseeable future but that the quality and quantity of it might alter over time.

- 33 The second defence approach was that even if there was a finding of continuing physical injury, the resulting impairment was not serious. Dr. Clifford felt that Mr. Delange could return to full physical activity, although he admitted that would be painful. Dr. Yovanovich felt that Mr. Delange could learn to live with his symptoms and that activity was unlikely to cause functional problems so that it would be safe for Mr. Delange to resume all of his activities.
- 34 All of the doctors recognize that Mr. Delange suffered physical injury. They all recognize he suffered not only a fractured sternum but also soft tissue injuries in the cervical and thoracic area. In the physical medicine - rehabilitation area, Dr. Finestone and Dr. Clifford, both with excellent credentials, disagreed on the prognosis. Although Dr. Clifford was a treating physician, in the sense that he saw Mr. Delange two or three times on referrals from his family doctor, I prefer the opinion of Dr. Finestone because his reports of the results of his physical examination coincide much better with the results of the physical examinations of the other doctors. Dr. Young, in my view, in his research into and report on the serratus muscle, has found a very cogent physical reason for the "band-like" pain that Mr. Delange has described. Dr. Tomen, the doctor who knows Mr. Delange best has concluded that his patient now has a chronic pain condition and that he does not see him as magnifying his symptoms. Dr. Yovanovich, in doing the defence medical noted Mr. Delange to be cooperative and forthright and sensed no evidence of magnification of symptoms.
- 35 There were no psychological opinions presented in this case and there was no evidence of any psychological examinations of Ray Delange having been undertaken. Dr. Clifford, in his discussion of a "valid belief system", seemed to be alluding to a possibility of purely psychological reasons for the continuing pain, and defence counsel pressed that theory in submissions.
- 36 I find there is no factual basis here for such a conclusion. The evidence here is of initial serious physical injury, and the "live issue" is whether the permanent symptoms are connected to that physical injury. While the evidence here raised the possibility (not probability) of some psychological component to the continuing symptoms, there is no evidence it is the source of the symptoms. I agree with Eberhard J, who found in *Dickson v. Canada Life Casualty Insurance Co.* (1996), 32 O.R. (3d) 175 (Ont. Gen. Div.) that physical injury does not have to be the exclusive cause of the alleged permanent impairment, and adopt her reasoning.

- 37 Dr. Tomen spoke of Mr. Delange as seeking treatment and relief, and very cooperatively undergoing every treatment that was available, without success. Mr. Delange has described his frustration at his inability to resume his previous high level of activity, and he has been supported in that by his wife and a number of people who have worked with him.
- 38 In his evidence in chief and during his cross-examination, Mr. Delange appeared to me to be forthright and honest, not exaggerating his difficulties, both physical and in relation to his business. He was unshaken in his cross-examination. I have no reason to doubt his testimony.
- 39 I have carefully considered the medical evidence. None of the doctors disagree on his very evident sternum fractures or on the whiplash type cervical injury. There may be some disagreement as to whether the pain and discomfort in the shoulder level arises from the injury to the chest or the injury to the neck, but there is no doubt it is there and related to the accident. As above noted, I feel Dr. Young's description of the serratus muscle provides an explanation for the band-like pain in the lower thoracic area. The doctors questioned about it did not disagree although Dr. Finestone felt there might be more than just that one muscle involved in the chest area.
- 40 I accept the evidence of the x-ray findings of degeneration in the low back, and Dr. Yovanovich's conclusion that the low back pain, first reported several years after the accident, is caused by that degeneration and is not accident related.
- 41 It is apparent that the sleep derangement, which is continuing, relates back to the accident, and was somehow caused by it.
- 42 My conclusion is that Mr. Delange has sustained a permanent impairment of the bodily functions in his neck, shoulder and chest area, extending, as indicated in the courtroom, to a point a few inches above the belt, with the injuries continuing and being physical in nature. I further find that the bodily functions of the upper body, which the medical evidence indicates are permanently impaired, are important, in that this affects nearly all aspects of the working and leisure life of Mr. Delange.
- 43 The final question is whether the impairment is serious. I note here that while Mr. Delange had at times referred to the chronic pain as like a toothache, he at other times had referred to it as discomfort or as a pulling sensation. It obviously is not excruciating or even severe pain. However some aspects of it are constant, and other aspects, while not constant, are constantly recurring.
- 44 While it is physically possible for Mr. Delange to do most if not all the things he did before, on an occasional basis, the point and his problem is

that he cannot do them constantly and consistently as he did before. As it came out in his evidence, he can clearly walk, but can no longer walk a mile, or even a kilometre. In view of his previous level of activity, working from six in the morning till nine at night, the impairment is indeed serious. This was put in *Lento v. Castaldo* at page 158 "... we are of the opinion that the frustration of an injured person's chosen career path generally should be considered to be a serious matter." As was said at page 142 of *Lento v. Castaldo* "an impairment of an important bodily function which is serious to one person may not necessarily be a serious one for someone else. The task of the court in each case will be to decide whether the impairment is serious to the particular injured person who is before the court. In performing that task the question will always be the detrimental effect which the impairment has upon the life of the particular injured person."

45 A recent example of the application of that is found in *Miranda v. Tenant* (1996), 31 O.R. (3d) 88 (Ont. Gen. Div.) where MacFarland J., in dealing with symptoms of headaches, dizziness, fatigue and difficulty in sleeping and concentrating following a closed head injury concluded "the record satisfies me that her injuries have substantially interfered with her abilities as a home-maker and care-giver to her son, which were her functions in life before the accident. These may not be important and serious functions for everyone but they were and are for this plaintiff and they have been substantially interfered with as the result of her injuries."

46 Here the evidence clearly indicates that there has been substantial interference with the work and leisure activities of Mr. Delange, which were important and serious functions for him. I find that the injuries sustained by Mr. Delange in the accident fit the exception of s. 266(1)(b) of the *Insurance Act*.

General Damages

47 Mr. Delange is entitled to damages for pain, suffering and loss of enjoyment of life. His evidence, well supported medically, is that the initial fractures to the sternum would have been extremely painful. That initial phase has been followed by his continuing and now chronic cervical and thoracic pain and sleep disturbance. The problem with the pain is not that it is unbearable but that it is unremitting. He describes it as a pulling sensation, a discomfort, something like a toothache, that it is with him on a daily basis, day in and day out, year after year. The pain, plus the chronic sleep disturbance leaves him constantly fatigued, and has changed his personality from outgoing and jolly to withdrawn and irritable. Despite his efforts, he is unable to work as he did before. Although some of the doctors expressed a faint

hope that his symptoms might lessen with time, they are still with him, and no one has said they will ever go completely away. His symptoms have impacted adversely on his career plans and on his leisure activities. He is a man who should be in the prime of his life and who finds himself chronically disabled. I assess his general damages at \$45,000.

Per Quod Servitium Amisit

48 The plaintiff proposes to claim for past and future income losses on the basis of the cost of replacement services. This is an approach that was approved in *Engel v. Kam-Ppelle Holdings Ltd.* (1993), 99 D.L.R. (4th) 401 (S.C.C.). The problem here is that Mr. Delange's heavy construction business was not a proprietorship, it was a corporation. The contracting business was carried on by Delway Contractors Ltd. The uncontradicted evidence was that Mr. Delange was the sole shareholder and sole director of the corporation. Delway Contractors Ltd. is not a plaintiff in this action. Mr. Greenaway argues forcefully that the loss being claimed by the plaintiffs is the loss of the limited corporation and not the loss of Mr. Delange. His argument is that the type of claim being put forth should have been put forth as a *per quod* claim and points to *Genereux v. Peterson Howell & Heather (Canada) Ltd.* (1972), 34 D.L.R. (3d) 614 (Ont. C.A.) Mr. Shulgan for the plaintiffs points out that the difficulty with a *per quod* type action is that *D'Amato v. Badger* (1996), 137 D.L.R. (4th) 129 (S.C.C.) and previous like cases in Canada appear to clearly prohibit a claim in a tort action for pure economic loss. As Major J. put it in *D'Amato* at page 139: "if a company is allowed to recover pure economic loss arising from the loss of a key shareholder and employee, the problem of indeterminacy arises and injury to one person obviously has a ripple effect, causing economic loss in various forms to a large number of people, both individuals and corporations. To allow recovery in these circumstances would invite similar claims by multi-membered plaintiffs."

49 Mr. Shulgan's argument is based on general principles - that it is perfectly obvious that Mr. Delange, as the sole owner and sole director of the company is the one who would gain if the corporate income went up, and was the one that would lose if corporate income went down, through an increase in expenses or otherwise. Certainly Mr. Strezos, a manager with Arthur Andersen & Co., a chartered accountant and a chartered business evaluator, called as an expert on behalf of the plaintiff, had no difficulty in expressing the professional opinion of that because Mr. Delange was the sole owner, losses would be attributed back to him from an income stand-

point, so that he could treat any increase in expense to the company as an expense of Mr. Delange.

50 Mr. Shulgan had little legal authority to support his position. He pointed to *Vaccaro v. Giruzzi* (1992), 93 D.L.R. (4th) 180 (Ont. Gen. Div.) where Potts J. discussed *Genereux* and *per quod* actions in relation to one man corporations. Potts J. concluded that *Genereux* should not be ignored in the case of a one person corporation, and specifically concluded that while the corporation in *Vaccaro* was entitled to claim for replacement cost of an employee, it could not claim for loss of profits. In that case the issue was not before the court of whether the sole shareholder could make the claim personally.

51 The only case which Mr. Shulgan could refer me to in Ontario that is directly on point is *Korenicky v. Arrow Leasing Ltd.* (1972), 28 D.L.R. (3d) 59 (Ont. Co. Ct.), a decision of Ball, County Court Judge, in which the loss of income claim was for \$2,000. In that case Mr. Korenicky owned 98% of the stock of a plumbing and heating corporation with his wife and niece owning 1% each. Ball, J. reviewed several other decisions in Canada and in England, and concluded at page 63 that "the courts are treating this matter realistically. In the case of Mr. Korenicky, the company was simply his *modus operandi* in the business world. He controlled the company in every way. He was paid a weekly sum of \$100 for which no claim was made, as the figures mentioned are exclusive of this management salary. Its loss was his loss in every way. I therefore think he can recover the damages I have assessed at \$2,000 as part of his personal special damages."

52 I note that the problem of the one man corporation in the business world was considered in relation to insurance in the case of *Kosmopoulos v. Constitution Insurance Co.* (1983), 42 O.R. (2d) 428 (Ont. C.A.) where Zuber J.A. concluded that the sole shareholder of a corporation has an insurable interest in the corporation's assets. If the sole shareholder has such an interest in the assets of his corporation, for all the reasons set out by Zuber J.A. surely the sole shareholder has a similar interest in the income and expenses of his corporation such to provide a proper legal basis for a claim for extra expenses incurred by his corporation due to his injury. If the position of the defence was accepted, it would leave Mr. Delange in the position where neither he nor his corporation would be entitled to claim for very real losses which arose directly from this accident, for purely technical and legalistic reasons. My conclusion, like that of Ball J., is that the courts are treating these matters realistically. I find, for the reasons aforesaid, that Mr. Delange is entitled to claim for the additional expenses incurred by his business in the past and anticipated to be incurred in the future.

Past Monetary Loss

53 The evidence of Mr. Delange and his other witnesses was that because of his partial incapacity, he had to hire others to take over some or all of his supervisory duties in the field. There was no contingency plan in place in the business, so for several days after the accident, work simply stopped. To get things going again various subcontractors, apparently at first on a most informal basis, simply pitched in to get the jobs moving. That eventually got regularized with various people covering from time to time as supervisors. Mr. Tim Cook for instance says that he spoke to Mrs. Delange on the morning of the accident, and offered to help for awhile and found he was there for two years.

54 Mr. Strezos, the expert that was retained by plaintiff's counsel, prepared a detailed appendix to his report showing "incremental subcontracting costs" totalling \$275,679 to September 30, 1996. Mr. Strezos in his testimony noted that he was dependent on the Delange family to tell him what costs had been incurred. He saw a number of invoices, some of which were illegible and appreciated that these were invoices from people who would have been doing subcontracting work for Mr. Delange's business in any event, but who were now doing more because they were called on to help as supervisors. Mr. Strezos had no way of independently verifying these figures, but he did do an analysis in which he took all of the subcontracting costs in each year for all of the years from 1990 through 1996, found the subcontracting costs had gone up substantially after the accident, but then backed out the figures given him for supervisory work and found then that the average cost of subcontracting before and after the accident was relatively stable.

55 The evidence of Tim Cook, Jim Ciliska, William Dawson and Richard Woudwyk, all of whom helped with supervisory work, was of little assistance on details. All recalled doing and charging for supervisory work. All also did other things for Ray Delange. They had not clearly differentiated in their billings what was for supervisory work and what was for other service. They could not now make that differentiation, and their "off the cuff" estimations from the witness box differed markedly from the figures used by Mr. Strezos.

56 Mr. and Mrs. Delange, on whom Mr. Strezos relied in preparing the detailed schedule of annual supervisory costs, were also of little assistance at trial. Mrs. Delange, although she had been handling the books, was not asked about details of the supervisory work. Mr. Delange knew supervisory work was done, and who had been doing it, but was very vague on exactly what was done, and at what cost.

57 The court is faced with a very detailed schedule of losses to November 30, 1996, of \$275,969, which is admittedly based upon estimates and interpretations, without the solid detailed underpinnings one would expect. Quite possibly, Mr. and Mrs. Delange, and those who worked for them, did not anticipate that details would eventually be needed. However it is, of course, up to the plaintiffs to demonstrate the loss. Clearly, there was some loss, and it becomes the duty of the court in this situation to attempt, from the information available, to hit upon a figure that would fairly and reasonably represent it.

58 My conclusion is that from the evidence and the figures, clearly more supervisory assistance was needed in the first two years than in the next five. I do not think the meagre facts herein would justify any complicated year by year analysis, or the imposition of any precise discount factor.

59 I fix upon \$80,000 for the first two years, and \$120,000 for the next five and a half years, up to trial, totalling in all \$200,000, as a fair and reasonable assessment of the monetary loss up to the trial date.

Future Monetary Loss

60 The evidence all indicates that Mr. Delange will continue to suffer disability, and that, other things being equal, the need for supervisory assistance will continue. Of course, all other things are not equal. Mr. Delange is engaged in a particularly volatile industry, as the construction statistics, and the variation in the gross and net profits of his business from year to year, found in Appendices to the Strezos report amply indicate. However, despite the ups and downs, and despite his own disabilities, Mr. Delange has managed to keep the business profitable. The most important contingency is the probable date on which he would have retired had the accident not intervened.

61 The most likely scenario, from the evidence presented, is that Mr. Delange would not have sold the business and suddenly retired. Rather, he has two sons whose formal education, and whose interests are directed towards this type of business. On the balance of probabilities he would have, and indeed probably still will gradually turn the business over to his sons. That would involve his gradually phasing himself out, rather than suddenly retiring. Mr. Delange was born on July 4, 1949. He is now 48. The evidence was that he had no fixed retirement plans before the accident, but had a goal of having the boys take over when he was 55 to 60 years old.

62 The future loss claim is not based directly upon the loss of income, but rather, as was the past loss, upon the extra costs involved for supervisory

help. It appears logical to me that when Mr. Delange's sons become involved in the business, they will assume the supervisory tasks now assigned to various subcontractors, even before Mr. Delange himself starts to withdraw. That assumption of work would not have been needed if Mr. Delange had not been injured, and as presumably the boys will have to be paid, it will not eliminate the expense, but it should lessen it.

- 63 I have been given a present value factor of 4.7085 to age 55, now roughly 7 years away, and another factor of 13.286 to age 65. Both factors include mortality as the only contingency. The only evidence of pre-accident plans was to turn over the business between 55 and 65 as a planned retirement age.
- 64 In my view, I am faced with the same problems as with the financial loss claim to trial, now further compounded by uncertainties as to future supervision costs, the uncertainty of a turnover and the date thereof, as well as, among others, contingencies such as economic and business reverses, and the possibilities of further physical recovery as well as of further accidents or illness. None of these lend themselves to exact mathematical calculation.
- 65 In the end, bearing all these factors in mind, I arrive at \$100,000 as a figure which to my mind, on the balance of probabilities represents a fair and reasonable assessment of the present value of the future monetary loss.

Liquidated Damages Hold-Back

- 66 Included in the report of Mr. Strezos, and thus in evidence before me, is a claim for \$4,300, being the rounded amount of a liquidated damaged charge-back re a contract. Mr. Strezos says he understood that to be uncharacteristic of Mr. Delange, and included it in his report as possibly attributable to the accident. I received no viva voce evidence, and no submissions on this point. There could be many reasons, quite unrelated to the accident, for such a charge-back. I take that part of the claim, if indeed it was ever intended to be part of the claim, as not being proved on a balance of probabilities.

Family Loss Claims

- 67 Mrs. Delange testified as to the effect of the accident on Mr. Delange, and the resulting changes in their social and personal lives. It is evident that the effects will probably continue indefinitely. She has clearly suffered a loss, damages for which I assess at \$10,000.

68 The two sons did not testify. There was some evidence from Ray Delange of the effects of his injury making him harder on the boys, but on the other hand he now has more time to spend with them. I do not find that losses have been proven in relation to the sons.

Conclusion

69 In summary, I find the defendants 100% at fault, find the claims are not barred by the *Insurance Act*, assess the general damages of Ray Delange at \$45,000, find him entitled to claim for monetary losses, assess those to date at \$200,000, and for the future at \$100,000, and assess the F.L.A. claim of Mrs. Delange at \$10,000 and dismiss the F.L.A. claims of Andrew and George Delange. If needed, re quantum of benefits to be deducted, prejudgment interest, costs, or any other matter, a further date can be arranged with me through the trial co-ordinator.

Order accordingly.